



PROCEDURE	
Sexual Assault – Reporting and Management of	
Scope (Staff):	All NMHS Mental Health (MH) Staff
Scope (Area):	All NMHS Inpatient, Community and Specialities MH Services

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1. Aim

This procedure outlines the requirements for the reporting and management of patients and consumers who report a recent sexual assault or unwelcome behaviour whilst within a NMHS Mental Health (MH) inpatient unit or whilst under the care of a NMHS MH community/specialities service.

If a worker or other person reports a sexual assault or unwelcome behaviour within the workplace, they must be offered wellbeing assistance as soon as practicable and encouraged to lodge an incident report with Health Safety and Wellbeing (HSW) for support. This can be done confidentially via direct email or phone call to HSW if the worker or other person wishes, there are resources to assist with reporting assault to Police and dealing with unwelcome behaviour in the workplace.

2. Background

Sexual assault is a broad term used to describe a range of sexual acts committed against a person without their consent. It occurs when a person is forced, coerced, or tricked into sexual acts against their will or without their consent. NMHS MH will strive to ensure patients/consumers are managed in a safe environment to prevent incidents of sexual assault. In the event sexual assault does occur against a patient/consumer, workers will report and manage the incident following the principles and guidelines in this procedure.

3. Risk

This procedure is required to reduce the potential severe clinical risk of sexual assault, harassment, and abuse within NMHS MH services. Supporting and promoting sexual safety to prevent the occurrence of these events provides protection and promotes safety for patients/consumers.

Non-compliance with this policy will impact on the following risk categories:

Culture	☒	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☐
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☐	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

4. Definitions

Sexual Assault: Sexual assault is a broad term used to describe a range of sexual acts committed against a person without their consent. It occurs when a person is forced, coerced, or tricked into sexual acts against their will or without their consent.

Sexual Assault Resource Centre (SARC): Is a free 24-hour emergency service based at King Edward Memorial Hospital providing crisis services to people who have experienced a recent sexual assault. Services are available for people of all sexualities and gender identities aged 13 years and above.

Sexual Safety: Sexual safety refers to being and feeling psychologically and physically safe, including being free of, and feeling safe from, behaviour of a sexual nature that is unwanted, or makes another person feel uncomfortable, afraid or unsafe. This includes sexual assault and harassment. It also extends to being spoken to using sexualised language or observing other people behaving in a sexually disinhibited manner, include nakedness and exposure or masturbation, being made to watch or shown pornographic images and lacking privacy and dignity when naked.

Sexual harassment: Sexual harassment is any behaviour that is deemed offensive including inappropriate remarks or jokes, discriminatory behaviour, unwanted romantic advances, gestures, or unwelcome behaviour. It can include a single isolated incident.

5. Sexually Safe Environment

A sexual safe environment is:

- Trauma informed, gender sensitive and emphasises recovery.
- Balances personal autonomy and decision making with a duty of care to provide a safe and therapeutic environment for all consumers.
- Recognises the common needs of individuals for privacy and personal space, and that these needs vary between individuals, regardless of gender identify or sexual orientation.
- Recognises the need for routine identification of sexual risk in all consumers who receive care from mental health services.
- Recognises the vulnerability of some consumers due to past history, illness, emotional turmoil or need.
- Promotes treatment and early recovery, and actively manages potential disruptions to treatment and wellbeing as a result of breaches of sexual safety.
- Promotes personal self-care, respect, resilience and self-determination.
- Responds sensitively to disclosure of past or current sexual assault or abuse.
- Listens and responds sensitively and appropriately to all concerns raised about sexual safety.

6. Principles

NMHS MH has a duty of care and legal obligation to provide health care in an environment that promotes safety and minimises risk of inappropriate behaviour, including that of a sexual nature, allegedly perpetrated by patients, staff, students, contractors, volunteers, and visitors. All patients/consumers have a right to engage in care and treatment in a safe and suitable environment/way.

NMHS MH services foster a compassionate, sensitive, and open culture that encourages open and transparent incident reporting relating to the sexual safety of patients/consumers.

NMHS MH staff have a responsibility to take action to prevent and appropriately respond to sexual safety incidents/assault.

All patients/consumers who report a sexual assault during an episode of care are to be treated according to the WA Health "[Procedure for responding to a recent sexual assault, DOH 2017](#)", the [Chief Psychiatrist's Guidelines for the Sexual Safety of Consumer of Mental Health Services \(OCP Sexual Safety Guidelines\)](#), [MP 0121/19 Responding to the Abuse of Older People \(Elder Abuse\) Policy](#) and '[Guidelines for Protecting Children 2020](#)'.

Both the alleged victim and the alleged perpetrator are to be treated with respect and dignity, and both parties provided with information and support where relevant and required in relation to the alleged sexual assault, providing support around their mental health and providing psychosocial support.

Service provision must respect and be inclusive of cultural, religious, sexual orientation, gender identity and potential language needs of the patient/consumer and the patient/consumer should be advised of their rights to access interpreters or other support services if required. If interpreters are requested, they should be provided during all stages of the process.

All alleged recent sexual assaults must be reported to the Office of Chief Psychiatrist as a 'Serious Event' and on the Clinical Incident Management System (Datix CIMS) as per Section 254 Mental Health Act WA 2014.

Comprehensive and contemporaneous documentation should be recorded about the incident immediately by clinical workers once reported; the patient/consumer's expressed wishes including reporting, the process of advising parents, carers, family, personal support person etc. and the actions taken. Specific details of the alleged sexual assault should not be included within the patient/consumer's medical records. Each service must ensure local protocols are in place for separate files for this purpose as is the required practice for complaints records as per the [WA Health Complaint Management Policy \(3.7 Recording of complaints\)](#). However a reference must be made in the patient/consumer's medical record to the existence of a separate confidential file.

7. Procedures

The relevant sections of the [WA Health responding to an allegation of sexual assault disclosed within a public mental health service guidelines 2012](#) are to be followed in relation to:

- The provision of information to patients/consumers and workers, available from the Sexual Assault Resource Centre ([SARC](#))
- Immediate response checklist (Refer to **Appendix 1** – Patient and **Appendix 2** Consumer within MHS)
- Clinical procedures for the care of the alleged victim and the alleged perpetrator (Section 2.2)
- Medical and Forensic considerations (Section 2.3)
- Administrative Procedures, including the reporting to the Chief Psychiatrist (Section 2.4)
- Clinical considerations, including documentation requirements (Section 3.1)
- Management of sexual assault victims within an inpatient unit (Section 3.2)
- Repeated allegations of sexual assault (Section 3.3)
- Patients/consumers who are psychotic or display acutely disturbed behaviour (Section 3.4)
- Involvement of WA Police (Section 3.5)
- Sexual contact between workers and patients/consumers (Section 3.6)
- Addressing Diverse needs such as interpreter access for Aboriginal and Torres Strait Islander, Culturally and Linguistically Diverse and hearing-impaired patients/consumers; accessing Disability Services Commission for patients/consumers with a disability; accessing appropriate support workers when working with older patients/consumers; and access to a Guardian for patients/consumers with impaired capacity. (Section 4.0)
- Patients/consumers under 18 years of age (Section 4.7)
- Worker support and debriefing (Section 5.1) – Also refer to NMHS Policy for [Hazard/Incident Reporting and Investigation](#)
- The provision of training and information about sexual assault/abuse/safety for staff. (Section 5.2)
- Confidentiality, documentation, and record keeping (Section 5.3)
- Mandatory reporting requirements (Section 5.4)
- Cross Agency collaboration and liaison (Section 5.5)
- Support for patients/consumers, family members, carers, and significant others (Section 6.0)

Any disclosure of a sexual assault complaint made against a NMHS worker is defined as serious misconduct and should be referred to the NMHS Integrity Directorate in line with the Notifiable and Reportable Conduct Policy, Discipline Policy and Health Services Act assess who will assess the matter, identify any risks, and provide advice and management in accordance with legislation.

8. Post Incident Debriefing and Support for Workers

Please refer to the [NMHS Critical Incident Response: Post Incident Debriefing Checklist](#).

8.1 Ongoing Support for Workers

Following the post-incident debriefing, managers should monitor and identify workers that may be adversely affected and initiate and maintain appropriate supportive action which may include:-

- One-to-one support from their supervisor or manager;
- [EAP](#) referral, in addition to debriefing sessions;
- NMHS Health Safety and Wellbeing Department for Injury Management consultation or [Fitness for Work Assessment](#); or
- Referral back to their own treating professional, for example GP or Mental Health Professional.

9. Compliance and Evaluation

- 100% of incidents are reported immediately to the Inpatient Clinical team (Psychiatrist/senior nurse in charge and reported to the Office of Chief Psychiatrist and recorded on Datix CIMS by CNS/CN by end of shift.)
- 100% of incidents within Community Mental Health and Specialities are reported immediately to the appropriate senior clinician by the end of the working day and recorded on Datix CIMS
- 100% of all incidents have been investigated by an appropriate person to ensure compliance with reporting and investigation process.

Related internal policies, procedures, and guidelines

[Responding to an Allegation of Sexual Assault Disclosed within a Public Mental Health Service Guidelines, 2012](#)

[Guideline for Responding to Family and Domestic Violence, 2014](#)

[Chief Psychiatrist's Guidelines for the Sexual Safety of Consumer of Mental Health Services](#)

[MP 0121/19 Responding to the Abuse of Older People \(Elder Abuse\) Policy](#)

[Guidelines for Protecting Children 2020](#)

[MP 0130/20 DoH Complaints Management Policy](#)

[NMHS Guide – Reporting Workplace Violence & Aggression Incidents to WA Police](#)

[MP 0166/21 Mandatory Reporting of Child Sexual Abuse Training Policy](#)

References

[Sexual Assault Resource Centre \(SARC\)](#)

[Mental Health Act 2014](#)

Useful resources

[SARC Education and Training](#)

[SARC Care Package - For adults who have experienced sexual trauma](#)

[SARC Supporters Guide – For people supporting an adult survivor or sexual trauma](#)

[SARC Supporting someone who has been sexually assaulted](#)

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Policy Contact	Co-Director Adult Inpatient					
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NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 2: Partnering with Consumers ☐  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety			☐  Std 5: Comprehensive Care ☐  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☐  Std 8: Recognising and Responding to Acute Deterioration		
Chief Psychiatrist's Standards for Clinical Care:	Physical Health Care of Mental Health Consumers,					
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11/11/2019	2.0	Clinical Nurse Specialist SCGH Mental Health Service	Scheduled review, updated template, policy reviewed and amended to meet current standards and reflect legislative requirements, NSQHS standards and hyperlinks added
09/10/2023	3.0	Co-Director Inpatients	Major review, update of reporting and management requirements, immediate response checklists updated in line with current practice for Inpatient and Community Mental Health Services

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative

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Appendix 1: Immediate Response for an Alleged Recent Sexual Assault within an Inpatient Mental Health Unit (including Adult, Older Adult and Forensic)

✓	Immediate Action Required:
	Ensure the consumer's safety and privacy and risk management, safety and privacy of the alleged perpetrator if they are an inpatient.
	Assess the psychological well-being and immediate physical and safety needs of the consumer and/or the person reporting the alleged assault.
	Head of Clinical Service (HOCS) & Coordinator of Nursing are to be notified immediately (via the Emergency Management On Call roster if out of hours). The treating psychiatrist or delegate, must contact The 'Sexual Assault Resource Centre' (SARC) 24 hour crisis line (08) 6458 1828 or 1800 199 888 who will provide guidance or a forensically trained medical officer and counsellor to assist with advice regarding informed consent to forensic examination, if required, and to consent to police involvement if necessary.
	Allocate an appropriate worker to remain with the consumer to provide support and care.
	If required, arrange for an appropriate translator or interpreting service for the consumer
	Advise the consumer that they are entitled to access a support person of their choice and facilitate contact
	Provide relevant SARC information required (available via: Sexual Assault Resource Centre)
	Advise the consumer that they are entitled to speak with a doctor, have a general medical examination and receive advice and medical treatments such as contraception and information on prevention of sexually transmitted infections (STI) prevention if appropriate.
	Advise the consumer that they have the right to police involvement if they so choose, which might include a forensic examination (consider social worker support)
ALLEGED PERPETRATOR (if patient/consumer, relative, staff member or visitor:	
	Ensure the immediate safety and welfare of the consumer and advise them of the allegation in an appropriate manner. If the consumer is an involuntary adult, a voluntary or involuntary child, consideration should be given to contacting the Mental Health Advocacy Service (6234 6300) who will offer to advocate for the consumer.
	If the consumer remains within close proximity to the victim, allocate an appropriate staff member to remain with them and ensure that their rights and privacy are not violated in the process. Consider arranging additional security or depending on bed

	availability transfer to alternative ward. Develop a safety plan to mitigate further risks.
	Determine if the consumer requires an interpreter and arrange for an appropriate person to be available throughout the process, including during any medical examinations or police interviews.
	Assess physical and emotional well-being and action where required including assessment for suicide, homicide, and interpersonal violence risk
	Advise of rights in relation to accessing medical advice, appropriate acute forensic testing and legal advice.
	Advise the Police if there was reasonable suspicion of impairment to decision-making capacity
	If a staff member, the staff member was taken to a private space away from the alleged victim and provided with support and assured confidentiality. Escalation to the relevant Department head must occur and consideration of police contact from their position. The Department head would be the central point of contact for further liaison regarding the staff member.
	Refer the consumer to appropriate support services.

Appendix 2: Immediate Response for an Alleged Recent Sexual Assault within Adult Community/Specialities Mental Health Service

✓	Immediate Action Required:
	Ensure the Individuals immediate safety and listen to the disclosure
	Assess immediate physical and safety needs of the Individual and/or the person reporting the alleged assault and call ambulance and police if appropriate and with consent.
	Assess the psychological well-being of the Individual and/or the person reporting the alleged assault.
	Ensure Service Coordinator or Team leader, Head of Clinical Service (HOCS), are to be notified immediately. Seek advice from the 'Sexual Assault Resource Centre' (SARC) 24 hour crisis line (08) 6458 1828 or 1800 199 888. SARC will provide guidance or provide a forensically trained medical officer and counsellor to assist with assessing the individual's capacity to give informed consent to forensic examination and to consent to police involvement if necessary.
	Allocate an appropriate staff member to remain with the individual to provide support and care as appropriate
	If required, arrange for an appropriate translator or interpreting service for the individual
	Advise the consumer that they are entitled to access a support person of their choice including cultural appropriateness and facilitate contact
	Provide relevant SARC information required available via: Sexual Assault Resource Centre
	Encourage the individual to speak with a General Practitioner
	Advise the individual that they have the right to a forensic examination and police involvement if they so choose
	Ensure appropriate notifications are made as required
ALLEGED PERPETRATOR if they are a consumer of Adult Community/Specialities Mental Health Service	
	Assess physical and emotional well-being and action where required including assessment for suicide, homicide, and interpersonal violence risk
	Allocate two appropriate staff members to remain with them and ensure that their rights and privacy are not violated in the process. Develop a safety plan to mitigate further risks.
	Determine if the individual requires an interpreter and arrange for an appropriate person including culturally appropriate supports to be available.

	Refer the individual to appropriate support services.
	Advise of how to access rights in relation to accessing medical advice, appropriate acute forensic testing, and legal advice, as per SARC advice
	If individual is under the Mental Health Act, ensure notifiable event to Office of Chief Psychiatrist is completed
ALLEGED PERPETRATOR if they are a staff member of Community Mental Health Service	
	Advise the staff members direct line manager who will follow NMHS Policy for Hazard/Incident Reporting and Investigation